

Banner Health

Business Development Dossier

Industry	Healthcare System
Revenue	\$14.1B
Employees	55,000
Ideal Customer Profile Score	88

ICP SCORING MODEL



ICP = Signals + Revenue + Employees + Fit (max 100)

Signals (0-35)	Top 4 signals scored; each = base(60%) + recency bonus <30d: 40% <90d: 30% <180d: 20% older: 10%
Revenue (0-25)	\$500M-\$10B sweet spot -> 70-100% of 25 pts Below \$500M -> pro-rata x 0.5 Above \$10B -> 60%
Employees (0-20)	2,000+ preferred (70-100%) 500-2K partial (50-70%) Below 500 -> pro-rata x 0.3
Fit (0-20)	High-fit types: reorg, transformation, M&A, hiring, funding, partnership -> 5 pts ea (max 12) + diversity 2/type (max 8)

Prepared March 26, 2026

McChrystal Group

Leadership Advisory & Organizational Effectiveness

SECTION 1

Organization Snapshot

Legal Name	Banner Health
Headquarters	Phoenix, Arizona
Founded	1999
Ownership	Nonprofit 501(c)(3) organization
Footprint	

Operations across six states: Arizona (primary), Colorado, Wyoming, Nebraska, Nevada, and Northern Colorado. 33 hospitals (30 acute care, 3 rehabilitation). Largest employer in Arizona

SECTION 1b

Company Overview

Banner Health is one of the largest nonprofit healthcare systems in the U.S., operating 33 hospitals across six states with \$16 billion in revenue and 55,000+ employees. Under CEO Amy Perry (since mid-2024), the system is executing an 'accelerate' mandate: \$1 billion five-year technology investment (including Anthropic AI partnership for BannerWise), transformation from fee-for-service to value-based care, and the innovative Banner|Aetna 'payvider' joint venture covering 1.2 million lives. Despite strong financial momentum (\$444M operating income, 2.8% margin in 2025), Glassdoor reviews reveal significant cultural challenges ('fear-based,' 'disconnected upper management,' 3.5/5.0).

SECTION 2

Financial Health & Growth Stage

Understanding Banner Health's financial position and trajectory is critical to framing any engagement conversation.

Revenue: FY 2025 \$16.0B (up from \$15.6B in 2024, \$14.1B in 2023)

Operating income: \$444M (2.8% margin) in 2025, up from \$362M (2.3%) in 2024 - consistent improvement

Net income: \$1.45B in 2025

Technology investment: \$1B committed over 5 years

Pressures: Rising labor costs, Colorado market restructuring (351 workers displaced), leadership role eliminations, Medicaid/Medicare volatility

Tailwinds: Fast-growing Arizona population, Banner|Aetna gaining traction, AI/technology leadership, University of Arizona partnership, Fortune Most Innovative Companies (3 consecutive years)

SECTION 3

Leadership Team Profiles

The following contacts are recommended as primary outreach targets - champion-level leaders who feel the organizational pain daily and can sponsor McChrystal internally.

Adrienne Moore, DrPH - SVP, Strategy & Growth [PRIORITY TARGET]

Previously SVP Finance at Banner (9 years); 15+ years at Adventist Health. Doctorate in Public Health (Loma Linda). Degrees in healthcare management, finance, accounting

Outreach rationale: Brand new role created to drive growth strategy - defines how Banner grows and transforms. Needs cross-functional alignment to execute. Her strategic mandate touches every part of the organization

Julie Ann Alvarado-Dubek - EVP, Chief Administrative & People Officer [PRIORITY TARGET]

30+ years senior HR leadership. Previous: Fender Musical Instruments, Freescale Semiconductor, Motorola. Leads 'Banner experience' for 55,000+ team members

Outreach rationale: Owns culture for 55,000 employees during restructuring, leadership role elimination, and CEO transition. Glassdoor reviews cite HR leadership issues - she's at the center of the organizational challenge

Additional leadership team members relevant to the engagement:

Amy Perry - President & CEO

Previously Banner Health President & COO (joined November 2021). Named Modern Healthcare's 100 Most Influential (2 consecutive years)

Michael Reagin, CHCIO, MBA - EVP, Chief Technology Officer

27 years IT/digital healthcare leadership. Previous: CIO Sharp HealthCare, SVP CIO Sentara Health, CIO Cleveland Clinic Abu Dhabi, CTO Providence Health. Banner's first-ever CTO

Don Stanziano, MHA - SVP, Chief Marketing Officer

Previously CMO at Geisinger; 16 years at Scripps Health as VP Marketing. Career includes congressional press secretary and TV/newspaper journalist

Staci Dickerson - EVP, Chief Financial Officer

Previously SVP Finance/CFO at Sharp HealthCare. EY and Arthur Andersen background

SECTION 4

Organizational Culture & Structure

Culture and structure are often where McChrystal Group finds the deepest leverage. The following signals indicate how Banner Health operates today and where friction exists.

Organizational Structure

Recently restructured regional leadership - consolidated Arizona West/East into 'Arizona Community Delivery.' Eliminated SVP Partnership role after 10 months; created new SVP Strategy & Growth. Six-state footprint with regional leadership structures

Culture Signals

Glassdoor: 3.5/5.0 (4,162 reviews); only 56% recommend. Culture 3.4/5.0 - notably below peers

Key concerns: 'Went from amazing family culture to cold you-are-just-a-number culture.' 'RIFs becoming the norm at least once a year to meet margins for bonuses.' 'Upper management seems very disconnected.' 'HR senior leadership is amongst the worst - creating a toxic fear-based culture.' 'Always understaffed, low pay, excessive work.'

Positives: Good benefits, collaborative unit-level culture, focus on development and succession

Restructuring events: Colorado realignment (McKee ED closed Nov 2025, 351 workers displaced); SVP Partnership role eliminated after 10 months (Aug 2025); 'relatively small' number of leadership positions eliminated

McChrystal fit: Severe gap between leadership's transformation ambition and frontline reality. New CEO attempting culture change while simultaneously restructuring and eliminating roles - exactly the challenge McChrystal addresses

SECTION 5

Recent News & Trigger Events

The following developments from the past 18 months create urgency and provide natural conversation entry points for engaging Banner Health.

1. 2024-06-01 - Amy Perry becomes CEO after Peter Fine's 24-year retirement

Foundational transition - entire leadership culture must evolve after longest single-leader tenure

2. 2025-07-01 - First brand campaign in 8 years: 'It's Another Banner Day'

Brand repositioning from 'access and convenience' to 'clinical excellence' - requires organizational capability to deliver

3. 2025-08-01 - SVP Partnership role eliminated after only 10 months (Mark Garvin departure)

Rapid role creation/elimination signals organizational model isn't settled

4. 2025-11-01 - Colorado restructuring: McKee Medical Center ED closes; 351 workers displaced

Restructuring creates organizational trauma - workforce trust eroded; remaining employees need cultural rebuilding

5. 2025-12-01 - BannerWise AI platform deployed to all 55,000+ employees (Anthropic partnership)

Enterprise-wide technology deployment requires massive change management across six states

6. 2026-02-02 - Adrienne Moore named SVP Strategy & Growth (newly created role); Shelby Arveson named SVP Finance

Organizational strategy function rebuilt from scratch - strategic direction still being defined under new CEO

SECTION 6

McChrystal Fit Assessment

CEO transition (2024), active restructuring, \$1B tech investment creating organizational stress, severe Glassdoor signals, clear buyers (Moore, Alvarado-Dubek), \$16B revenue, organizational urgency (culture crisis + transformation + growth mandate simultaneously)

Primary Problem

New CEO executing ambitious transformation (\$1B tech investment, VBC pivot, brand repositioning) across 55,000-person, six-state system with significant cultural challenges (fear-based HR, disconnected management, annual RIFs) and leadership instability (role eliminations, rapid exec hiring/departures)

Best McChrystal Capability Fit

Culture transformation at scale; organizational design and stabilization; leadership alignment for almost-entirely-new C-suite; cross-functional coordination for Banner|Aetna payer-provider integration

Likely Objections to Engagement

We're already investing \$1B in technology transformation
We've been restructuring - change fatigue
Our Glassdoor scores are what they are

Competitive Advisory Landscape

Anthropic (tech partner, not consulting competitor)
McKinsey/Deloitte likely on strategy and transformation architecture
Huron Consulting for operational performance
Korn Ferry/Spencer Stuart for executive search
No competitor for 'culture transformation + operating model redesign during active restructuring'

SECTION 7

Conversation Entry Points

These questions are designed for a Senior Partner's first conversation. Each opens a thread that naturally leads to McChrystal Group's capabilities without a direct pitch.

1. You're asking 55,000 people to absorb a \$1 billion technology investment, a value-based care pivot, and a brand repositioning - simultaneously. How is the leadership structure at regional and facility level keeping pace with transformation?

Gets at disconnect between C-suite ambition and frontline reality

2. Banner|Aetna is one of the most interesting 'payvider' models in the country - but making clinical and insurance sides work as one requires a different operating model than either would need alone. How are you breaking down that boundary?

Cross-functional coordination using Banner's own innovative structure

3. You created a new SVP Strategy & Growth role after eliminating the SVP Partnership role - which suggests organizational design for how Banner drives growth is still evolving. What's the vision for how strategy, operations, and regional leadership align?

Directly addresses organizational design instability

Recommended Meeting Framing

'We work with organizations attempting exactly what you're doing - transforming culture, technology, and operating model at scale. We'd like to understand how you're thinking about the organizational model that makes your \$1 billion technology bet pay off.'

SECTION 8

Brand Insights & Market Positioning

Brand strategy reveals organizational priorities and coordination challenges. The following analysis connects Banner Health's market positioning to potential McChrystal engagement opportunities.

Brand: Largest AZ employer, one of largest US health systems (\$16B). Fortune Most Innovative Companies (3 years). However, brand historically known for 'access and convenience' not clinical excellence.

Evolution: 'It's Another Banner Day' campaign (summer 2025) - first in 8 years, shifting to clinical excellence positioning. Brand evolution parallels organizational transformation - promising something the system is still building. CMO Stanziano's 'glocal' strategy (global consistency + local relevance) mirrors the multi-state coherence challenge.

Threats: Colorado closures risk community trust; Glassdoor scores (3.5, fear-based culture) undermine employer brand; Arizona competitors (HCA, CommonSpirit, Abrazo) investing in market position; Banner|Aetna dual identity risks consumer confusion.

McChrystal connection: Marketing says 'clinical excellence and innovation.' Glassdoor says 'cold, fear-based, disconnected.' Brand-organization gap is unsustainable - employees who feel devalued can't deliver what the brand promises. McChrystal bridges this.

SECTION 9

Deep McChrystal Group Fit Analysis

This is the strategic case for pursuing Banner Health. It synthesizes findings from all previous sections into a comprehensive engagement thesis with specific fit dimensions, expected outcomes, stakeholder mapping, and a phased pursuit strategy.

9a. Fit Dimensions

Dim 1: Post-CEO-Transition Culture Transformation - Peter Fine led 24 years. Perry attempting shift from Fine's legacy to tech-forward, VBC model. Glassdoor: 'cold,' 'fear-based,' 'disconnected upper management.' McChrystal

specializes in culture transformation in organizations with broken trust. Korn Ferry coaches individuals; McChrystal changes how leaders at every level operate.

Dim 2: Multi-State Operating Model Coherence - Six states, recently restructured regions. Shared consciousness across diverse markets (urban Phoenix, rural Wyoming, competitive Colorado) while allowing regional autonomy. McKinsey redesigns on paper; McChrystal builds human coordination.

Dim 3: Payer-Provider Integration (Banner|Aetna) - 50/50 JV requiring unprecedented coordination between clinical ops and insurance ops. McChrystal's cross-functional alignment directly applicable. Different incentive structures, professional cultures, operational rhythms.

Dim 4: Technology Transformation Change Absorption - \$1B investment requiring 55,000 employees across six states to change how they work. Fails for organizational reasons, not technology reasons. McChrystal builds capacity to absorb transformation.

Dim 5: Organizational Design Stabilization - Created and eliminated senior roles in months, restructured regions, closed facilities. Serial reorganization signals model isn't settled. McChrystal helps arrive at stable, effective design.

9b. Cumulative Case

Fine retirement -> Perry CEO (mid-2024) -> entirely new C-suite -> \$1B tech investment -> SVP role created/eliminated in 10 months -> Colorado restructuring/351 displaced -> first brand campaign in 8 years -> VBC acceleration -> Glassdoor 'fear-based culture' -> culture crisis + transformation ambition = maximum tension.

Revenue: Phase 1: \$750K-\$1.2M (exec alignment + culture diagnostic) | Phase 2: \$2.5M-\$4M (Team of Teams across six states) | Phase 3: \$500K-\$1M/year | Total: \$3.75M-\$6.2M

9c. Enterprise Issues

1. Cultural debt from 24-year leadership era ('fear-based,' 'you are just a number')
2. Almost entirely new C-suite without established operating rhythms
3. Serial organizational design without stabilization
4. \$1B tech investment without proportional organizational change investment
5. Payer-provider silo in Banner|Aetna model
6. Brand-culture disconnect ('clinical excellence' vs. 'fear-based culture')
7. Colorado market trauma (facility closures, 351 displaced)
8. Value-based care transition organizational immaturity

9d. Expected Outcomes

1. Executive team operating effectiveness - decision cycle time reduced 35-45%; eliminate serial reorg pattern
2. Six-state cultural transformation - Glassdoor Culture from 3.4 to 4.0+; 'would recommend' from 56% to 70%+
3. Payer-provider coordination for Banner|Aetna - membership growth acceleration; friction reduced
4. Technology organizational readiness - BannerWise adoption >80%; clinician admin reduction on track
5. Colorado market recovery - engagement scores recovered to system average within 12 months
6. Regional leadership alignment - cross-regional coordination improved 30%
7. Stable organizational design - no unplanned role eliminations within 18 months

9e. Key Stakeholders

CEO Office

Leader: Amy Perry

Title: President & CEO

Relevance: Executive sponsor - 'accelerate' mandate

Strategy & Growth

Leader: Adrienne Moore, DrPH

Title: SVP

Relevance: PRIMARY TARGET - new role, new mandate

People & Culture

Leader: Julie Ann Alvarado-Dubek

Title: EVP CAPO

Relevance: PRIORITY TARGET - culture owner, Glassdoor signals

Technology

Leader: Michael Reagin

Title: EVP CTO

Relevance: \$1B tech investment needs org readiness

Marketing

Leader: Don Stanziano

Title: SVP CMO

Relevance: Brand-culture alignment ally

Finance

Leader: Staci Dickerson

Title: EVP CFO

Relevance: Budget holder

Banner

Leader: Aetna

Title: [JV leadership]

Relevance: JV CEO

Payer-provider coordination

9f. Opportunity Thesis

Why Now: Perry 18 months in - mandate but still early. Almost entirely new C-suite. \$1B tech investment. Glassdoor culture crisis. Colorado restructuring trauma. Brand campaign promising what culture can't deliver. Maximum tension = maximum receptivity.

Why McChrystal: Banner invested in strategy (McKinsey), technology (Anthropic), talent (Korn Ferry). Nobody invested in the ORGANIZATIONAL OPERATING MODEL that makes all other investments work. The structural paradox: invested in everything except the connective tissue.

Phased Engagement: Phase 1 (3-4 months, \$750K-\$1.2M): C-suite alignment + culture diagnostic | Phase 2 (10-14 months, \$2.5M-\$4M): Team of Teams across six states + Banner|Aetna coordination + Colorado recovery |

Phase 3 (\$500K-\$1M/year): Culture transformation advisory

Pursuit Map: Thread 1: Adrienne Moore in her first 90 days (Feb-May 2026). Thread 2: Julie Ann Alvarado-Dubek via CHRO networks. Thread 3: Amy Perry via Modern Healthcare 100 Most Influential network + military connections (Luke AFB, Fort Huachuca).